

1. Adenoiditis aural symptoms are NOT:

- a) Tubal obstruction ✓
- ☒ b) Sinusitis
- c) Recurrent attacks of acute otitis media ✓
- ☒ d) Nasal discharge
- e) Serous otitis media ✓

2. The epiglottis is formed by:

- a) Muscle
- b) Bone
- ☒ c) Fibrous cartilage
- d) Adipose tissue
- e) Skin

3. Which of the following statements are INCORRECT:

- ☒ a) In Weber test the tuning fork is placed on the mastoid process *Rinne - test*
- b) In Weber test a patient with normal hearing localizes the tone either in the center of the head or equally in both ears
- c) In Weber test a patient with unilateral conductive hearing loss localizes the tone in the affected ear
- ☒ d) In Weber test a patient with neurosensorial hearing loss localizes the tone in the affected ear
- e) In Weber test a patient with neurosensorial hearing loss localizes the tone in the healthy ear

4. Differential diagnosis of acute tonsillitis is performed with:

- ☒ a) Aphthous ulcers
- ☒ b) Malignant tumor of the tonsil
- c) Sinusitis
- ☒ d) Buccopharyngeal manifestations in Agranulocytosis
- ☒ e) Buccopharyngeal manifestations of Leukemia

5. Otosclerosis is characterized by:

- a) Eustachian tube function is impaired
- ☒ b) tinnitus
- c) hyalinisation and calcification of subepithelial connective tissue, white chalky deposits in the promontory and ossicles
- ☒ d) conductive hearing loss
- ☒ e) paracusis Willisii

6. Chronic non specific laryngitis can NOT be:

- a) catarrhal
- ☒ b) caused by tuberculosis
- c) hypertrophic-red and white type
- ☒ d) atrophic
- ☒ e) caused by syphilis

7. Waldeyer's ring is composed on the following elements, with one exception. Indicate the exception:

- ☒ a) Nasopharyngeal tonsil or the adenoids

- b) Palatine tonsils or simply the tonsils
- ☒ c) Submaxillary glands
- d) Tubal tonsil (in fossa of Rosenmuller)
- e) Nodules (in posterior pharyngeal wall)

8. Regarding pharynx anatomy which of the following statements is incorrect:

- a) Narrowest portion (1.5cm) is at its caudal end
- b) It extends from base of skull to inferior border of cricoid cartilage anteriorly and inferior border of C6 posteriorly
- ☒ c) The pharynx has more than 20 cm. in length *12-14cm*
- d) Widest portion (5cm) is at hyoid level
- e) The pharynx is divided into 3 parts: nasopharynx, oropharynx, and laryngo(hypo)pharynx

9. Waldeyer's ring is NOT composed on the following elements:

- a) Nasopharyngeal tonsil or the adenoids
- b) Palatine tonsils or simply the tonsils
- ☒ c) Gastric tonsils
- ☒ d) Mastoid tonsils
- e) Tubal tonsil (in fossa of Rosenmuller)

10. In acute mastoiditis

- a) Treatment consists of stapedotomy
- ☒ b) Treatment consists of antibiotics
- c) Treatment consists of reconstructive surgery with hearing restoration by tympanoplasty
- ☒ d) Treatment consists of myringotomy
- ☒ e) Treatment consists of cortical mastoidectomy

11. Regarding oropharyngeal functions the following answer is incorrect:

- a) Conduit for passage of air or food
- b) Helps in pharyngeal phase of deglutition
- ☒ c) Helps in appreciation of the smell
- d) Provide local defense and immunity against harmful intruders into the air and food passages
- e) Forms part of vocal tract for certain speech sounds

12. Abscesses in relation to mastoid infection are:

- ☒ a) Bezold
- ☒ b) petrositis } *google*
- c) zygomatic
- d) orbital cellulitis
- ☒ e) postauricular *safe*

13. Symptoms of acute tonsillitis are:

- a) Oral breathing
- ☒ b) Difficulty in swallowing
- ☒ c) Fever
- ☒ d) Sore throat
- ☒ e) Earache

old lecture!

*infectious mono
Vincent's angina
traumatic ulcer
diphtheria*

*not do
tympano
sclerosis*

hears better in noisy env

*specific type
of laryngitis*

*+ fetid breath
pallor hyperemia*

14. The hypopharynx is clinically subdivided into:

- a) Vocal cord
- b) Ventricular fold
- ☒ c) Pyriform sinus
- ☒ d) Post-cricoid area
- ☒ e) Posterior pharyngeal wall

15. About the nasal skin, which affirmations are correct?

- a) thin skin with many sebaceous glands that covers the nasal bones and the lower lateral cartilages
- b) thin and adherent skin, covering the alar cartilages
- c) thick skin rich in sebaceous glands that covers the nasal bones and the upper lateral cartilages
- ☒ d) thin skin and freely mobile, that covers the nasal bones
- ☒ e) thick, adherent and rich in sebaceous glands skin, that covers the alar cartilages

16. Larynx benign tumors are:

- ☒ a) Chondroma
- ☒ b) Osteoma
- ☒ c) Haemangioma
- d) Botriomycoma
- ☒ e) Papilloma

17. Regarding Juvenile Nasopharyngeal Angiofibroma the following statements are correct:

- ☒ a) Appears progressive nasal obstruction and nasal speech
- b) The treatment is always surgical
- c) Age and sex (exclusively in males in the age group of 10-20 years)
- ☒ d) The exact cause is unknown
- ☒ e) Biopsy of the tumor is associated with profuse bleeding and is, therefore, avoided

18. The posterior epistaxis:

- a) the most common cause is trauma
- b) mostly occurs in children or young adults
- ☒ c) originates mostly from posterosuperior part of nasal cavity, often difficult to localize the bleeding point
- ☒ d) occurs spontaneous often due to hypertension or arteriosclerosis
- e) is usually mild and easy to control

19. Congenital lesions of the larynx are:

- ☒ a) Laryngomalacia
- b) Choanal atresia
- ☒ c) Laryngeal Web
- d) Cleft palate
- ☒ e) Laryngocele

20*. Nasopharyngeal functions are the following, with one exception. Indicate the exception:

- a) Drainage channel for the mucus secreted by nasal and nasopharyngeal glands
- ☒ b) Passage for air and liquids
- c) Ventilates the middle ear through Eustachian tube
- d) Resonating chamber during voice production
- ☒ e) Conduit for air (warmed and humidified)

21. Which of the following are acute tonsillitis complications:

- ☒ a) Acute glomerulonephritis
- ☒ b) Subacute bacterial endocarditis
- ☒ c) Rheumatic fever
- d) Convergent strabismus
- ☒ e) Chronic tonsillitis

22. Differential diagnosis of acute tonsillitis is performed with:

- a) Rhinitis
- ☒ b) Vincent's angina
- ☒ c) Infectious mononucleosis
- ☒ d) Diphtheria
- ☒ e) Membranous tonsillitis

23. Regarding hypopharyngeal malignant tumors, which of the following statements are correct? all the lecture

- ☒ a) Prognosis is poor
- ☒ b) Mirror examination may not be useful
- c) The most frequent site of origin is subglottic region
- ☒ d) Progressive dysphagia, malnutrition and weight loss, change of voice and disphonia
- ☒ e) Surgery, Radiotherapy, and combined treatment

24.* The nasal septum divides the nose into:

- a) superior and inferior nasal cavity
- b) lateral and medial cavity
- c) four cavities 2 superior and 2 inferior
- d) the nasal septum is virtual
- ☒ e) right and left nasal cavity

25.* The olfactory nerves, (12-20) nerves which pass through the cribriform plate and end in the:

- a) olfactory lobe
- b) parietal lobe
- c) temporal lobe
- ☒ d) olfactory bulb
- e) frontal lobe

located posterior to larynx.

testosterone dependant growth.

also cryotherapy or embolisation

anterior

after 40yrs

severe, req hospitalization

*laryngeal cyst
laryngoesophageal cleft
subglottic stenosis
hemangioma*

26. The rhinophyma:

- a) appears due to the hypertrophy of the perspiratory glands of the tip of the nose
- ☒ b) is a benign, slow growing mass
- ☒ c) is treated with surgical excision or with laser Co2
- ☒ d) is a slow growing malign tumor
- ☒ e) appears due to the hypertrophy of the sebaceous glands of the tip of the nose

27* The maxillary sinus:

- a) opens into the superior meatus
- b) its innervation is from the IX-th cranial nerve
- ☒ c) it is the largest sinus
- d) opens into the superior meatus
- e) has a 5 ml capacity ~15

28. Nasal symptoms of vasomotor rhinitis are NOT:

- ☒ a) nasal obstruction ✓
- ☒ b) bronchospasm
- ☒ c) postnasal drip ✓
- ☒ d) wheezing
- ☒ e) ocular prurit

29.* A dermoid cyst is:

- a) an infection
- b) a blood collection
- c) a malign tumor
- d) a sign of consumption
- ☒ e) a congenital tumor

30.* Into the middle meatus open the following sinuses:

- a) maxillary, frontal and sphenoid
- b) posterior ethmoidal sinuses
- c) sphenoid sinus
- ☒ d) anterior sinuses
- e) pyriform sinus

31. The septal haematoma:

- a) is a collection of connective tissue
- ☒ b) incision and drainage in larger ones with bilateral nasal packing
- ☒ c) is a collection of blood under the perichondrium or periosteum of the nasal septum
- ☒ d) it can be caused by nasal trauma or septal surgery
- ☒ e) the treatment consists in aspiration with a wide bore sterile needle

32. The rhinoscleroma:

- ☒ a) its clinical stages are: atrophic, granulomatous, cicatricial
- b) its etiology is gram + Staphylococcus Aureus
- ☒ c) its etiology is gram negative bacillus - Klebsiella Rhinoscleromatis
- ☒ d) its certain diagnosis is given by biopsy

☒ is surgically treated

Antibiotics
streptomycin + tetracycline

33. Allergic rhinitis treatment consists of:

- ☒ a) antihistaminics
- b) supraexposure to allergen ~ der verrech
- ☒ c) corticosteroids (oral and topical)
- ☒ d) avoidance of allergen
- e) antibiotherapy

+ immunotherapy !

34* The first step in the management of anterior epistaxis is:

- ☒ a) pinching the nose with thumb and index finger for 5 minutes
- b) posterior nasal packing
- c) cauterization
- d) ligation of vessels
- e) anterior nasal packing

35. The choanal atresia:

- ☒ a) an important diagnostic sign is the failure to pass a catheter from oral cavity to oropharynx
- ☒ b) can be: uni-, bilateral, complete, incomplete, bony (90%) or membranous (10%)
- ☒ c) it is an emergency in bilateral choanal atresia
- ☒ d) one of the diagnostic signs is the absence of air bubbles in the nasal discharge
- e) it's always bilateral, complete, bony or membranous

36. The local causes of epistaxis are:

- ☒ a) foreign bodies
- ☒ b) trauma/
- c) cold
- ☒ d) infections
- ☒ e) neoplasms of nose and paranasal sinuses

37. The complications of frontal sinusitis are:

- ☒ a) osteomyelitis of frontal bone
- b) maxillary sinus necrosis
- ☒ c) meningitis, extradural abscess, or frontal lobe abscess
- d) acute conjunctivitis
- ☒ e) orbital cellulites

38. Choose the correct affirmations about laryngomalacia:

- ☒ a) Male predominance 2:1
- ☒ b) Flaccidity of supraglottic laryngeal tissues
- c) Most frequent cause of stridor in adults ~ children
- ☒ d) Characterized by inward collapse of pharynx supraglottic structures
- ☒ e) Most common congenital laryngeal anomaly (50-75%)

39. Subglottic haemangiomas:

- ☒ are reddish-blue mass below the vocal cords
- ☒ biopsy - strongly indicated *contraindicated*
- ☒ are symptomatic till 12 months *ascm till 3-6 mo*
- ☒ 50% have associated cutaneous haemangiomas
- ☒ stridor with ~~abnormal~~ *normal* cry

40. * The main indication for tracheotomy is:

- a) to treat malignancy
- b) to administrate anesthesia
- ☒ to bypass upper airway obstruction
- d) nasal obstruction
- e) to treat thyroid tumors

41. * The most common cause of bilateral recurrent laryngeal nerve paralysis is:

- a) A tumor mass expanding in upper part of mediastinum,
- b) Laryngeal trauma,
- c) Toxic polyneuropathy of recurrent laryngeal nerves,
- ☒ Surgical iatrogenic damage in case of total thyroidectomy due to thyroid gland carcinoma,
- ☒ Acute subglottic laryngitis.

42. Larynx functions are:

- ☒ thoracic fixation - *"stabilizes thorax"*
- b) expectoration
- ☒ phonation
- ☒ respiration
- e) inferior respiratory airway protection

43. Vocal nodules:

- ☒ are considered tumoral tissue reactions
- ☒ more frequent in male patients -
- ☒ are not favored by voice overload -
- d) are surgically treated -
- ☒ voice rest is recommended

44. Complications of laryngotracheal trauma are represented by:

- a) Loss of taste
- ☒ Perichondritis / Laryngeal abscess
- ☒ Vocal cord paralysis
- ☒ Laryngeal stenosis
- e) Hiposmia

45. Regarding Nasopharyngeal Cancer the following statements are correct:

- a) Neurologic symptoms has to be present at debut
- b) Distant metastases are into the Eustachian tube
- ☒ Cervical nodal metastases may be the only manifestation
- ☒ Age of onset: 5th to 7th decades, but may involve younger age groups

☒ Otologic symptoms and signs are represented by the conductive hearing loss, serous and suppurative otitis media

46. Laryngotracheal trauma can be represented by:

- ☒ Haematoma and edema of the larynx
- ☒ Othematoma - *Bei Kinden!*
- ☒ Dislocation of cricoarytenoid joint
- ☒ Fractures of hyoid bones, thyroid and cricoid cartilages
- ☒ Fractures of upper tracheal rings

47. Following are intracranial complications of otitis media

- a) petrositis
- b) labyrinthitis
- ☒ extradural abscess
- ☒ meningitis
- ☒ brain abscess

48. Following are intratemporal complications of otitis media

- a) subdural abscess
- ☒ facial paralysis
- ☒ mastoiditis
- d) meningitis
- ☒ petrositis

49. Following are signs of acute mastoiditis

- a) blood counts show monocytes and lymphocyte leucocytosis
- ☒ conductive hearing loss
- c) neurosensorial hearing loss
- ☒ mastoid tenderness
- ☒ purulent ear discharge

50*. The structures of oropharyngeal wall are the following, with one exception. Indicate the exception:

- a) Mucous membrane
- b) Pharyngeal aponeurosis (pharyngobasilar fascia)
- c) Muscular fascia
- d) Buccopharyngeal fascia
- ☒ Periosteum

↳ Bone in Wange? S